pHI guys:)) Pls write down what questions you rmb was asked during your CL orals session with lily and whatever you answered in blue

Good luck pookies!!! <333

Monday

Morning

9:30 am

- First question was to convert Rx to contact lenses: R -3.00/-2.25x130 L -2.00/-2.25x180. I changed it to -3.00/-2.00x130 and -2.00/-2.00x180 and she asked what was the rationale behind this so i showed her with power crosses. Then i said i would go for 2.25 cyl because that is whats available and then she asked why not 1.75. I said well in this case i might go for 1.75 because u dont wanna overminus their rx.
- Then she said what problems would u expect if the lens does not have great oxygen permeability and i went through all the layers:
    - Epithelial: microcysts, NV, oedema
    - Stroma: NV, striae, folds, haze
    - Endothelium: blebs, polymorphism, polymegathism
- She then asked what would lead to corneal scarring, and i mentioned chronic oedema can lead to thinning and eventually scarring
- She then moved on to the fit and she said What would you be assessing and i said movement centration coverage tightness. Then she asked why is coverage important and i said to make sure that we prevent staining and corneal desiccation. Then she went on to ask why is it important to cover the limbus and i mentioned that the limbus has stem cells and NV can start from there. She didnt probe any further on that.
- She asked what brand would u pick?I picked myday torics and she asked why those, and I mentioned the range of rx from +8.00 to -10.00 and the cyl availabilities, she asked about the axis availability for this specific rx and I said it's available for all 180 degrees in 10 step. I also mentioned the dk/t of 100 and low modulus so better comfort and oxygen permeability blah blah.
- For the fit she probed further on what else would i be assessing and i said the axis orientation and then she said where do u expect it to be and i said 6 o'clock, then she asked what would u do if it was rocking between 5-7 o'clock i said i would switch the brand.
- Then she asked about what i would do if the marking was at 5 o'clock i initially said LARS and subtract 10 degrees then she said r u sure it's 10 and i was like lol nvm its 30 degrees. And then she said where would the marking of the new lens be and i said 5 o'clock.
- Final question was, what would u expect to change about the lens with the new cyl axis. And i just mentioned the edge thickness would rotate 30 degrees as well (it was a confusing ass question i had to ask her 3 times what she meant)
- Good luck thats all

Absolute legend thank you!!!

10:20 am

- Only gave me R spec Rx (same as above; -3.00/-2.25x130) because “let’s start with one eye at a time”. Asked me to convert and tell her my thought process. What contact lens do you want to prescribe? And with which Rx
    - Told her -1.75 since -2.00 not available in cyl
- Ask her for more info on px: 31M office worker looking to replace specs, first time wearer, ochealth and gen health good, no hobbies, no signs or symptoms of DE
    - “Why did you ask these questions?”
- Which lens do you prescribe? I said MyDay toric daily disposable.
    - Why? I said comfort and explained parameters linked to that
    - “Any other reasons for choosing MyDay?”
- What are the other parameters/features of MyDay?
    - Blanked out and said “uv blocker…?”
- … Ok, so you mentioned dk/t earlier. What signs of corneal hypoxia?
- Asked something about what parameter is good for masking cyl (cos i mentioned the 0.25 diff from conversion) and said high modulus → stiffer lens
- Why a DD? When would you not consider DD?
    - She gave a big nod when I said px can demonstrate good CL hygiene.
- We trial the lens. What are you looking for on SLE?
    - Good coverage, centration, movement, tightness and minimal rotation
        - What are the values you expect of good fit?
- If it is too tight, what complications?
- If it is too loose, what are the complications?
- What is SEAL? How does it happen?
- Went back to SLE, what else do you want to check for?
    - Surface wettability. I only said rotation earlier, so she wanted me to specifically say marking location.
- I think that’s about it? So we never got to the left eye. Overall she seemed really patient and would guide you. 8/10 experience. Never again.

10:40am

- Similar rx to above: -3.00/2.25x135, -3.00/-1.75x180
- Converted to: -3.00/-2.00x135, -3.00/-1.50x180
- Said if choosing a lens like CV MyDay, choose -3.00/-1.75x130 (due to possible nasal rotation) and -3.00/-1.50x180.
- Pretty much the same as above (hypoxia, if the lens rotated, etc.) but asked me for a monthly lens too
- Asked what complications for too little movement and too much movement
- Asked about inflammatory CI complications
- Asked about solutions and why
    - What if they don’t have the schedule to soak lenses for 6h+
- Asked about what properties we consider when picking a CI (Dk/t, wettability, modulus of elasticity etc.)

11.00am

- DIFFERENT RX 😭
- Rx: R +5.00/-2.00x145 & L +4.00/-1.00 x 180
- Started by converting Rx to +5.25/-2.25 x 145 & +4.25/-1.25 x 180
- Showed her my working out & got into selecting a lens. Before this I asked H&S and she said 31 year old office worker & first time CL, OHx unremarkable.
- Selected MyDay Toric, mentioned that it's in 10 degree steps so changed RE to 140 due to nasal torque.
- What do you do once you fit the lens?

➔ Check VA, fit (centration, coverage, rotation, tightness, movement).

- What movement do we expect & why is limbal coverage important?

➔ 0.2-0.5mm & avoid indentation, limbus has stem cells for corneal regeneration

- Then she said RE turns 20 degrees temporally what do you do?

➔ Change 140 to 160 degrees.

- Do you expect marking to be in the same position? If it isn't then why not?

➔ I said yes. If not I said it's due to different thickness distribution & effects of gravity/blink force ???

- Next she said what patient advice would you give?

➔ I said hygiene, red flags for infection, building tolerance to lens wear.

- Asked me about going to the beach

➔ I said wear Rx goggles, she said they need CLs. I said okay goggles on top of DD & wait 15-20mins or irrigate with saline before removal due to lens adherence.

- She said what's lens adherence?

➔ Due to the tonicity difference CL can adhere. She then asked if it is the same in pool water ?!? I said I think it's worse in pool water cause it has higher tonicity she said okay??

- Then she said review schedule?

➔ 1 week, 1 month, 6 months & yearly afterwards cause they only DD wearer.

- She said what do you check at 1 week?

➔ VA, fit, evert lids/staining to check complications.

- She said why do you check fit again if we checked it on first visit?

➔ I said it's because they only wear lenses for a little bit in office. Wearing them for a week means it'll settle, dryness of lens can also change tightness/fit, she said okay.

- Then she said why evert lids?

➔ I said check if they have CLPC and if they have allergies it might be worse she said so is CLPC allergy only and I said it has dual mechanism immune response (deposits/allergens etc) & mechanical (CL is a FB).

- Then we said bye lol.

11:17

- +5.00/-2.00x145
- Conversion to +5.25/-2.25x145
- What lens are you choosing

- My day toric +5.00/-2.25x140 (for some reason i thought they only came in 0.50 steps above +4.00, i think this is a total 1 thing). Talked about parameters, 8.6 BC, 54% water content, stenfilcon A, one of the few that has axis 140 for -2.25 cyl
- Why daily disposable?
- Assuming they are first time wearer and they are not wearing it very often for special occasions. It might be better for their lifestyle and reduces the risk of severe complications. Less maintenance
- What are the pros and cons of a water content of 54%
- Comfort, but can lead to dry eyes and lipid deposition
- Why does higher water content cause lipid deposition?
- Cause it dries out the aqueous and leaves behind the lipid
- What are the considerations with modulus? Pros and cons
- Comfort and handling
- What else? Stares at me for 2 seconds
- Contact lens associated complications from high modulus such as SEAL
- What causes SEAL?
- Mechanical friction from upper eyelid against hard lens. She lets me go from this
- What are the complications with oxygen permeability, so why do you need high dk/t
    - Talked about anaerobic metabolism, lactate
    - Epithelial: microcysts, NV, oedema
    - Stroma: NV, striae, folds, haze
    - Endothelium: blebs, polymorphism, polymegathism
- You've listed a lot of signs, but why is it important?
- Can impair vision from scarring due to inflammation or deposition from pannus/neovascularisation of the cornea.
- What are you looking for once the lens is on the eye
- Vision, comfort, centration, coverage, movement, tightness
- You look at the lens and it is at 5 o'clock in right eye
- I mixed up my left and rights but Lily helped me out. So to the right, 30 degrees. Subtract to 110
- Once you chose new lens, where is the marking
- It should still be at 5 o'clock but i have adjusted the lens so the refractive axis is where I want it to be, 140.
- What might affect the fit after you changed the cyl? IDK? Think about the physical property of the lens.
- Iso horizontal thickness design for a continuous ballast. Hence changing the axis from an oblique one to a more against the rule has better fit.
- What advice are you giving a first time wearer
- No sleeping, showering, swimming. Avoid vegetative matter or dirty environments. Clean dry hands. Avoid tap water.
- When do you review this patient and what do you review
- 1-2 weeks for aftercare, they have tried the lenses. Make sure comfort, fit, and vision are still good. Forgot to mention lid eversion cry🥲.
- Why would the fit change

- Time to adapt unlike the 5-10minutes we had in the clinic. Might change by the end of the day, due to dry eyes.

11:50AM

- +5.00/-2.00x145, +4.00/-1.00x180
  I got +5.25/-2.25x140(5 degrees nasal torque), +4.25/-1.25x180
- What lens are you choosing?
  I would ask about age, motivation, occupation, and hygiene (to decide if they are a good candidate for monthly lens)
  Then i said ideally we wanna give daily lens, i would go with myday toric (didnt ask about parameter)
- Why is age important?
- Presbyopia
- What else?
  To check about general health diseases, such as diabetes, hypertension, and medication.
- How do u tell if px has good hygiene?
- I said i mainly observe their nails and lashes. She said ok
- Why do choose myday toric?
  Because myday toric has high water content and low modulus, it gives good comfort for first time wearer.
- What do u assess after fitting in lens?
- Lens coverage, lens movement, lens orientation, lid interaction
- Then asked what else?
- I blanked out,
- she said what they getting cls lens for?
- I said oh VA and over-refraction
- What is a good lens movement?
- 0.2-0.4mm movement
- What is a good lens coverage?
- I cant remember! Then i was like 0.4?, (correct answer is 1mm pass the limbus, and when u blink, it doesnt cross the limbus)
- What is the flat fit complication?
- Excessive lens movement - instability - toric marking is not stable - causing fluctuating vision. + lids interaction - can cause discomfort
- What about steep fit?
- Reduced tear film exchange - corneal hypoxia, conjunctival indentation, blur vision afterwards.
- How do u prescribe cls diameter? What is normal cornea size?
- I didnt revise this, correct answer is 11.5mm normal corneal size, and cls diamete would be HVID+2mm
- Let's say lens marking is sitting at 5 o'clock RE, what would u do?
- 30 degrees to the right of the examiner, then i would minus 30 from 140, then i would change the axis to 110

- After changing lens axis to 110, where is lens marking now?
- I said 6 (apparently it's wrong), it's at 5. But idk why
    - Its the same lens so you don't expect it to sit differently on the same eye, it may be rotated but the effective axis correcting the astigmatism will be 140.

Afternoon

12:08 pm - Rx +5.00/-2.00x145 and +4.00/-1.00x180

- Answer: +5.25/-2.25x145 (140 axis to account for nasal torque) and +4.25/-1.25x180

Guys most of my questions were same as above - Which lens? I asked about age and occupation she said why is that important

- My Day toric - good dk/t, wettability, lens parameters 10 degree steps for higher cyls included
- I said occupation good for record keeping/shift workers/lifestyle/lifeguards or water based + age for pre presb or presb

She asked what id look for when fitting lens

- VA, fit eg. centration, movement, etc.

She asked about lens movement whats adequate

- I said 0.2mm - 0.5mm is good

She said what if its less or more

- Too tight if less - hypoxia concerns?? (reduced tear exchange) → she prompted on hypoxia saying how and is tear lens exchange the only thing that provides oxygen. I said most oxygen comes from atmospheric + 5% from tarsal plate vasculature, tear lens is still important though and if reduced also less debris/deposit removal through reduced exchange
- Too loose - discomfort, possible dislodging, moving around eye = potential mechanical complications

Asked if its rotating in RE nasally 20 deg what do you do and what do you hope for after fit

- LARS minus 20 from axis so 120 degrees in RE, hoping to see axis marker at that accounted axis not 6 oclock,

She asked what are factors that contribute if its the lens is not at the intended axis you wanted it to be at after not accounting for it

- Manufacturers variation, stabilsation thickness areas are different in new axis, blink forces?

She then said when would you choose monthly and what solution would you picl

- I said id like to fit most of my patients initially in dailies unless parameters arent available or they come in wanting it specifically. Id ensure theyre compliant to hygiene/hygienic person in general/switched on.
- Picked
- moist, aldox property → strong acanthamoeba trophozoite killing activitiy (make sure you don't say it eliminates risk of acanth tho)

12:25 pm

- R +5.00/-2.00x145 L +4.00/-1.00x180
    - Answer R +5.25/-2.25x145 L +4.25/-1.25x180

- What lens would you give?
    - My day toric. Good Dk/t value of 80, low modulus for good comforts. For this rx daily toric, myday is a good option.
- What would you check in cl?
    - VA, fit (movement, lag, centration, axis mark, rotation), comfort
- What is a good movement and why is movement important?
    - 0.2-0.5mm.
    - For tear exchange, oxygen permeability
- What else?
    - Not sure. But now i think of it for comforts and to present any mechanical complications
- Fit is good. What would you do next?
    - Give trials, book after care after 1-2 weeks later. (forgot to mention teach)
- What would you check in after care?
    - Compliance, fit, va, comforts, ask about complications.
- Why check fit again?
    - More time for the lens to settle.
- What complications can u see in the after care?
    - SEAL, CLPC
- How does SEAL occur?
    - Poor fit. Localised dryness due to a poor tear exchange?
- In what kind of lens do you expect to see SEAL?
    - High modulus lens.
- Do you expect to see CLPU during after care?
    - Not really. Usually for long term wearers and extended wearers.
- Going back to cl rx, do u order the same rx?
    - Change RE to 140 degree as myday axis only comes in 10 deg increments. Nasal torque.
- RE lens rotate 20 deg nasally what would you do?
    - Order 120 degree. Then I corrected it to 130 as the original axis was 145. (she guided me to correct.
- Where do u expect to see the axis mark?
    - 6 o'clock. Then I corrected it to 20 deg nasally. (she guided me)
- When do you recommend monthly wear
    - Cost when px wearing every day.
- What else?
    - For extended wear. Eg. shift worker.

12:42pm

- Rx: R +5.00/ 2-.00 x 145 L +4.00/ -1.00 x 180. Can you convert into CL rx?
    - R +5.25/ -2.25 x 145 L +4.25/ -1.25 x 180. She was like you can talk me through what you're writing. I said I would not adjust axis yet cause depending on what CL i picked axis may be available
- What lens will you pick for this px?

    - I said MyDay toric, she asked why. I said it covers rx range and cyl and I said i would now change axis in the RE to 140 to cover for nasal torque bc 145 not available. She smiled and did not ask further on dk/t, modulus, etc.
- What do you assess of the CL?
    - I said coverage, tightness, movement, marking of toric. WHAT ELSE? I said horizontal lag. LILY WHAT ELSE? I said over-refraction and vision
- Where would you expect the toric lens marker to be?
    - Ideally 6 o'clock
- What will you do if the toric marker is at 5 o'clock for RE?
    - I said order new axis according to LARS rule. 5 o'clock to the right of the examiner so subtract 30 degrees. New axis is 110.
- Where would you expect the new lens marker to be?
    - I said 5 o'clock
- What are some reasons why it would not be at 5 o'clock?
    - Gravitational forces, forces from lid blinking? WHAT ELSE? Thickness of the lens, peri ballast in toric lens can cause the lens to shift.
- What is the ideal limbal coverage?
    - I said 1mm each side even with movement/ blinking
- What is the ideal movement?
    - I said 0.2 - 0.5mm
- What happens if the lens does not move?
    - Me: ?? Um I said px comfort, too tight can cause hypoxic changes? LILY WHAT ELSE? I said uh nothing coming to me head.
- Ok… what happens if the lens moves too much?
    - Excessive movement = increased lens awareness, fluctuating vision because toric marker going everywhere. WHAT ELSE? IDK??
- Lily probing me, tear exchange behind lens, what happens then?
    - Honestly I think she confused herself here. I said um tear exchange is good for oxygen?? She was like ok… but WHAT ELSE? I said uh protein deposition? She was like ok…. What happens to protein deposition? I was like ??? affects the ocular surface?? And then she was like ok I think this was a silly question LOLOL
- Px wants to use a reusable lens, what do you do and what lens will you pick?
    - I said monthly Biofinity Toric lens but I would first let the px trial DDs first to assess their lens compliance and hygiene before confirming whether I want to prescribe monthlies for them because increased risk of infection blah blah
- How do you assess their hygiene?
    - I said I looking at their CL wearing habits in DDs (eg. washing hands, discarding daily, not over wearing them, etc) and their eyelid hygiene and nails. She seemed happy with this answer.
- What solution would you recommended for the px?
    - I said AOSept but need to check what their schedule/ occupation is since it requires at least 6hrs to neutralise. Otherwise can do OptiFree PureMoist but need to teach them rub and rinse.
- Px wants to go to the beach, what will you do?

    - Ideally no CL wear to the beach but if px wants can wear cls with goggles on top. Must remove after exiting water (explained the whole hypotonic environment, wait 15mins or rinse with saline then remove otherwise will rip their epithelium off LOL)
- Can they wear for the lens for the rest of the day after going to beach
    - I said ABSOLUTELY NOT must remove ASAP!
- Then she said ok I think that's enough for today, have a great afternoon and smiled.

No one has any 1 or 2 o'clock sessions ?? 🥹🥹🥹
Is she on lunch, no clue
:(- arvo should start 1.40pm
So does no one have any sessions…..
When's everyone's sessionsssss I'm stresseddd
515 bro should i die, nah but i wanna get it over with
Wait fam u have heaps of time. Deadass no one has anything right now like seriously help

3.00PM hello 3pm please help, mine be coming up soon
Yall 1pm-2pm you guys didnt help with this doc 😭yall were gatekeeping

1. -5.00/-2.00 x 145 RE and -4.00/-1.00 x 180
   Lily said this is the spectacle prescription and their first time wearing contact lenses. Can you convert the prescription. (she will ask you to walk her through how you're converting)
   Converted: RE -4.75/-1.75 x 145 & LE -3.75/-1.00 x 180 (told her I dont know the contact lens im choosing yet, so this is just the conversion)
2. What lens would you choose?
   Said i would go for myday toric due to high dk/t and low modulus
3. Ask me what would i check in a contact lens fit
   Said centration, coverage, movement, lag, push up
4. She then asks me what would you consider as a tight fit for push up
   There was alot of back of forth here LOL
   I said it depends, because push up is very subjective so i would need to rely on movement as well. I initially said anything >50% then she said so 50% you would change the lens and i was like no not necessarily, i would change if its >65% and if theres less movement
5. Then she goes what else are you looking for in contact lens fit
   Then i said need to look for the toric lens marking
6. She said what would you order
   RE -4.75/-1.75 x 140 (accounting for nasal torque), LE -3.75/-1.00 x 180
   Then she was like are you sure -1.00, then i said omg i retract, i would do LE -3.75/-0.75 x 180
7. Then she asks where do you expect the lens marking to be
   Said ideally if everything works out with the prescription i would expect it to be at 6 o'clock
8. She then asks me if the lens is rotated nasally both eyes by 20 degrees what would you do

Said i would use the lars rule and account for this so i would order new trials as RE -4.75/-1.75x 120 LE -3.75/-0.75 x 20

9. Then she asks if the patient is 39 years old, what are some consideration you need for this patient
   Said this patient is a prepresbyope so approaching presbyopia age, asked for binocular status (she said no dont have that info) so i said, could still fit the patient with these lenses but let them know that they are approaching the age where they may find it difficulty with near tasks and when they do they may need to fitted with a different contact lens
10. Do you expect the 39yr old to find more difficulty with near tasks earlier with contact lenses or glasses
    (FMD this fucking threw me off - but lily guided me towards the end, and you can take back things you say) but correct ans: since they are a myope they already need more accommodative and convergence demand with contact lenses therefore they will experience difficulty with near tasks earlier with contact lenses.
11. When would you review this patient
    Said i would do fit and teach and if that is successful give the patient the trials to try for one week and see them then, recheck fit and comfort
12. Why do you need to recheck fit
    Told her in office trial is limited due to consultation time but giving them the trial they can try for the whole day and the fit and comfort rating will be more accurate
13. Asks me so what are some reasons that can affect the fit
    Said tear film can be affected if the patient has dry eye, the patients occupation may also influence the patients rating of the comfort gave her an eg. office worker concentrating on computer and blinking enough or even environmental factors such as aircon drying out the contact lenses
14. What do you expect the lens fit to be after the aircon dries the contacts
    Expect the fit to be more tight since lenses are dried out
    Then she said good bye

3:15PM

1. -5.00/-2.00 x 145 RE and -4.00/-1.00 x 180, same rx as 3PM, asks me to convert and did the same and told me to walk her through it.
   Little bit of back and forth cause I was talking about consideration of the cyl and axis with available CL's and stuff.
2. Lens chosen?
   Said MyDay Toric, SiHy, low modulus, and fits in the range of the rx provided
3. Asked me about how would I assess the CL fit
   Spoke about va's, over rx, bv testing cause theyre myopic and so convergence and accommodation needs to be factored in, lens lag, movement, tightness and mentioned expected numbers. She then tried probing for extra stuff and then i spoke about lens centration and making sure its past the limbus by 1mm (i think) and mentioned to her lens marking cause toric lens

4. Asked how does the edge of CL if on the limbus affects limbus stem cells
   Kinda blanked out here and just spoke about how it affects a reduction in stem cell production of the cornea type thing
5. Asked me where I would expect the lens marking to be?
   I said around 6 O'clock ideally speaking.
6. Asked me about how I would change the axis if both CL's on the eye had their markings 20 degrees nasally.
   Spoke about LARS rule and how it would be RE: 120 and LE: 20.
7. Probed me more about providing the full rx for the RE.
   Just mentioned the full rx, kinda confused what she was trying to get at here but realised she just wanted the rx. It took me a while to get and tell her but then it was over.

3:30PM

1. Same rx as 3 and 3:15PM. Asked me to convert, explained how I used power cross + changes to effective power to vertex correct. Accounted for nasal torque in final CL rx.
2. Where do you expect the axis marking to be with 20 degrees nasal rotation. Will it change after you've ordered it accounting for LARS?
3. What lens did you choose? MyDay DD toric + explained the parameters
4. What happens with a higher vs lower water content lens
5. Whats the difference between a higher modulus vs lower modulus lens?
6. How would you assess CL fit? After VAs and over-rx, assess for centration (1mm beyond limbus), tightness (ideal = 40-60%), axis marking + rotational stability, lens movement (0.2-0.5mm), lens lag (0.2-0.4mm). Kinda just yapped so she wouldn't ask me anymore.
7. This px is 39 years old, what do you need to consider? Pre-presbyope to presbyope age, consider over specs, MFs, EDOFs
8. What do you need to think about with this px and MF/EDOFs? I said EDOFs will produce more glare, haloes, flare and reduced contrast sensitivity but i dont think thats what she wanted.
9. Asked me about whether you NEED to have this patient in MFs at the moment.
10. Name an EDOF brand. Seed pure 1 day
11. Name a MF, whats the power range. MyDay, shouldve said spherical range -10.00 to +8.00
12. I said some shit about convergence and accommodation changes as a myope (happy hyperopes miserable myopes), i dont remember what the q was LOL sorry
13. Px wants to swim, what do you tell them? Strongly advise against wearing CLs at all, push for prescription goggles. If they do want to wear CLs, they can wear DDs with air tight goggles. They must wait 10-15 minutes before removing lens or irrigate with saline before removing lens.
14. Why do they have to irrigate? The difference in tonicity between beach or pool water and tears = lens adherence = can rip off epithelium upon removal.
15. Said thats all we have time for today

5.00
-5.00/-2.00x135 : -2.00/-3.00x110

1. What is the CL rx and explain how you got there

R: -4.75/-1.75x130 L: -2.00/-2.75x110

2. What lens are you choosing
    a. I asked if they are first time wearer and how long they want to wear for
    b. She siad they are first time wearer want to replace their glasses - I said if they are first time wearer ideally DD so then i chose myday torics but they only come up to a -2.25 so the rest of the uncorrected cyl can be put as a spherical equivalent (-2.25/-2.25x110)
    c. Then she was like are there any DD that come with a 2.75 cyl - I said no (although i think there is a B+L one but i just dk parameters abt it so i left it 💀)
3. How are you assessing the fit of the contact lens
    a. Centration, lens lag, movement, tightness with push up, rotational stability then if the fit is acceptable then you can go to doing a VA and over-rx
4. How do you test for rotational stability?
    a. First want to locate the marking - then can move the CL off and watch if on blinking the lens returns back to its og position or not
5. What is the acceptable range of tightness on a push up test
    a. 40-60%
6. Why do you need good movement + what is the point of it ??
    a. Allow for some tear exchange to happen (even tho its minimal) + to allow for trapped debris to be removed?
7. What happens if debris are trapped?
    a. I said hypersensitivity to things under the lens or if it is microbes can increase risk of infection becuase its not getting flushed away
8. If rotating 20 degrees nasal OU what is your new axis
    a. If the rotation is stable then you can apply LARS
        i. R: 130 → 110
        ii. L: 110 → 130
9. If you fit the new CL where do you expect the marking to be
    a. I said still rotated nasally because youve accounted for the change by adjusting the axis
10. If they are a 39 year old what are your considerations
    a. I said work environment LOL (out doors = more prone to dust, indoors = dryness from airconditioning) then she was like what esle so then i said pre-presbyopic
    b. If shes pre-presbyopic what are solutions for her
        i. Over-rx specs, MFs, Monovision
    c. Are they going to show signs of being pre-presbyopic earlier
        i. Yes because they are myopic and they need to accommodate and converge more with contact lenses than with specs. Need to make sure that their near vision is adequate for the tasks they want to achieve
    d. If MF What wouuld you give them - i said biofinity toric MF, but it was a monthly lens which would require additional lens care; i.e. not ideal for a first time CL wearer

        i. And then she was like what are the risks? Of monthly vs daily disposable; there is the additional lens care -also greater risk of infection becuase you are not starting with a “fresh” lens so they would need to demonstrate good hygiene with clearing, storage and use of CLs.

11. What solutions would u give
    a. MPS
        i. How do you know what lens solution to choose - i was lowkey stumped,I said that they lens solutions usually indicat if they are suitable for RGPs or SOFTS, and then she was like yeh ok what else I said antimicrobial properties (but then she was like all shouldn have), so then i said some effective against acanthamoeba (but then she was like does it eliminate it) and i said well its not effective against the cyst form. So then i said that time it takes for the solution to soak also can factor what lens solution you use - i.e. Puremoist soak time is 6 hours whereas bioture is 4 hours - e.g. if the patient has certain time constraints due to lifestyle
12. Then she said thats all

5:15 👅

1. Spectacle rx: R -5.00/-2.00x135 L -2.00/-3.00x110. Convert to CL and tell me ur working
    a. R -4.75/-1.75x135 L -2.00/-2.75x110. Power crosses blah blah vertex correct
2. What CL would you choose and script and why
    a. I said My Day toric BUT I would have to spherical equivalent the left eye and account for nasal rotation of OD
    b. R -4.75/-1.75x130 L -2.25/-2.25x110. MyDay accounts for the script, then she cut me off so I couldnt yap about Dk/t and modulus
3. Why does the lens show nasal torque
    a. Bruh i dunno, i was just like it is what it is everytime the px blinks. Ig it has to do with stabilisation technique and eyelid forces
4. What are you looking for in contact lens fit Centration, movement, tightness, VA, Over-rx
5. Whats the acceptable movement - 0.2-0.4mm
6. Whats the acceptable coverage/centration - covers 1mm of limbus
7. Why do you want movement in contact lenses Need tear film to get under the lens for oxygen (prevent hypoxia) as well as remove any debris or microbial endotoxins that could be trapped under the lens
8. Where do you expect the lens marking to be - 6oclock
9. If the lens nasally rotates OU by 20 degrees whats the new axis Applying LARS R: 130 → 110 L: 110 → 130
10. Where do you expect the new lens marking to be - same rotation i.e 20 degrees nasally
11. If ur px was 39 what considerations are you thinking of - Presbyopia and struggling with near tasks, can swap to overspecs, MF, monovision
12. You mentioned overspecs, MF or monovision; what would you give and brand.
    a. MF - Lol i initially said Myday but then was like whoops they would complain hardcore about the cyl so then changed it to bioinfinity toric but then its a monthly rather than a DD

13. Why are u hesitant in giving px monthly (given that theyre a first time wearer)
    a. Yapped about hygiene in general and CL cases (more work) increased risk of inflammatory/infective responses. I also spoke about occupation and why they really need it considering this px is first time i.e if they only want on for special occasions than a DD is better realistically.
14. Do you expect the px to be more impacted by prebyopia earlier with contact lenses or with spectacles? CL, px is a myope so will have greater convergence and greater accomodative demand in CL compared to specs
15. If monthly what solution would you give? Optifree cause of polyquad (bacterial cover) and aldox (acanthamoebea trophozite) cover
16. What factors in a px lifestyle that would make you worried about acanthameobea (i forgot exact q) Spoke about constant water exposure i.e swimming, washing without drying hands and rinsing lenses with tap water
17. Say if ur px is consistently exposed to acanthameobea (what dat mean, r they swimming in a swap 24/7) is there any solution that can kill acanthamoebia I said if theyre swimming and exposed to all these pathogens all the time maybe they could try hydrogen peroxide but that also doesnt kill cysts - so i said not that i can think of
18. Say ur px wants to swim in CL, what advice would you give
    a. Dont do it fam - or wear googles. After swimming cause hypotonic difference, saline rinse and wait 15 min before removing CL, then dont use same CL for the rest of the day
19. Would you wear the same CL after swimming the next day, what would you do with them? OBV THROW EM OUT
20. What influences you to choose a specific MPS solution or hydrogen peroxide
    a. I mentioned allergies hydrogen peroxide has no preservatives after neutralisation vs MPS which does, time and conviencne i.e if they dont have it in them to wait 6 hours for the hydrogen peroxide to kick in whist MPS you can rub and rinse (although it should be in the case for 6hours)
21. What does rubbing and rinsing do Gets rid of any bacteria debris that is stuck on the lens that only rinsing cant get rid of

Tuesday

Morning
GOOD LUCK EVERYONE!!! PLEASE NO RGP DAY PLEASEEEE
Good luck!!!!

9:30am

1. 31 yo w spec rx R +6.00/-1.00 x 120, L +3.00/-2.00 x 110 ⇒ are they suitable for cl wear and can u convert to cl rx

Yes they can consider CL wear, R +6.50/-1.25 x 120, L +3.00/-2.00 x 110

2. Asked me what rx id order based on above

R +6.50/-1.25 x 120, L +3.00/-1.75 x 110

3. This px is 31 years old and a first time CL wearer, what type of CL would you recommend

Soft toric CL because first time wearer + convenience, if unsuccessful may consider RGPs

4. Which soft toric CL and why

CV MyDay Toric, wide parameter range + lower modulus (0.40) = softer = better for first time wearers to ease adaptation into CL wear/experience of having something on eye

5. Asked me about near work - what would CL wear do for this px visual tasks at near

Happy hyperopes ⇒ less accom + converg, also mentioned cosmetic benefit, no frame scotoma from spectacle wear

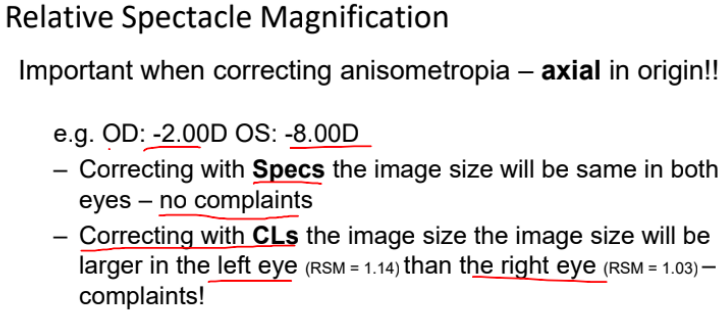
6. Asked me to comment on the rx in each eye, anything interesting going on etc.

Mentioned anisometropia (she made me clarify refractive or axial anisometropia) and I said refractive (idk if true couldn't remember) and just gave the rough aniso power in each meridian

Wait how do you know that this is refractive aniso and not axial? (generally, if the spherical anisometropia is ≥ 2 D, then it is likely to be axial) ok ok
as in you compare the sphere powers between the two eyes, or equivalent spheres? And then the toricity is most likely corneal shape?

7. Probed a bit more on the anisometropia aspect - specifically asked what happens to the magnification and retinal image size

Lowk panic a bit but said hyperopes lose mag through CLs and couldn't remember if ret image size was incr/decr so said incr but its wrong ffs (For axial aniso specs are better and for refractive aniso CL are better bc of the magnification diff between eyes + generally, if the spherical anisometropia is ≥ 2 D, then it is likely to be axial)



8. Moved onto fit assessment of the MyDay Torics ⇒ asked me what i'd look for

Same shit as yesterday basically just said 360deg limbal coverage of approx 1mm, movement, lag, tightness w/ PU, any lens rotation and also on-eye comfort

9. She then said lets assume everything is fine but the lens marking is showing 10deg temporal rotation OU, what do you do to the axis?

LARS ⇒ OD was changed from 120 to 130, OS was changed from 110 to 100

10. She said where do you expect the lens marking to be now

Same spot ish so roughly between 6-7 o'clock OD and between 5-6 o'clock OS (she guided me to say just 10deg temp again LOL)

11. She went back to RGPs ⇒ “you mentioned rigids at the start, what are some considerations for someone with this rx wearing RGPs”

High plus RGPs have a greater central thickness ⇒ can ride low due to gravity esp if lids are not tight to help w positioning, may show to excess movement

12. She asked how to correct it if riding low

Reduce bozr and increase LD

13. That's all we have time for

U did so well!!!
Wow good job! Why did lily go through literally almost every topic TOT am i anon badger
Was fighting for my life @anonymous badger
Wat animal am i jackalope
YOU'RE A LEGEND!

Has anyone else done their exam: TEXT THE CHAT
Omg where is everyone from the morninggg. Im 11.15
😬

10:38am - it was pretty much the exact same as the 9:30

1. First time CL wearer, same script as above - are they suitable for CL wear + convert to CL script.
    - Said suitable for SCLs as low corneal toricity but can consider RGPs due to high plus OD.
2. “Is the script you calculated the exact script you would order?”
    - Said yes because I lowkey forgot about the left eye being a -2.00 cyl but yes for the right eye.
3. What SCL would you give
    - MyDay Toric, went briefly into modulus but that was it, she didn’t really prompt for any parameters. Talked about how low modulus would be more comfortable for first time wearer but potentially more difficult to handle - if so then could change to a higher modulus lens like Alcon DT1.
    - So which one would you give?
        - Stuck to the MyDays lol
4. What are you looking for when you’re assessing fit.
    - Centration, movement, tightness, rotation, comfort, over-rx.

5. What is the ideal movement for an SCL? And where would you want the axis marking to be?
    - 0.2 to 0.5mm. 6 o'clock (mentioned that MyDay marking is at 6 o'clock but you probably dont need to specify)
6. Why do you want movement in a SCL?
    - Allows for some sort of tear exchange to occur to reduce hypoxia risks + allow some removal of debris.
7. Lenses are both rotated 10 degrees temporally, how would you change the axis.
    - 130 RE, 100 LE as per LARS rule.
8. Where would you expect the axis marking to be if you ordered that axis.
    - 10 degrees temporally still.
9. When would you consider a monthly lens over a daily one?
    - Lowkey i blanked because in my head i was like "...never?" but eventually I said if they had to use it more often and for longer periods and she nodded and moved on so ig that's fine.
    - Mentioned that before giving a monthly lens, you'd assess if the patient has good enough hygiene and motivation to keep the lenses clean for re-use.
10. What benefits would this patient get out of wearing contact lenses over spectacles.
    - Talked about reduction in anisometropia effects potentially, no peripheral scotoma due to base-to-base prism effect and cosmesis.
11. Going back to RGPs, what issues might this patient have with RGP wear.
    - Mentioned the low-riding right lens due to a combination of thick tear lens + thick lens centre thickness.
    - Also dropped VLK, dellen and 3 and 9 o'clock staining but she didn't probe into this at all.
12. If the lens was low riding, what could you consider changing about the lens.
    - Steepen base curve, increase lens diameter.
    - What else?
        - Mentioned maybe adjusting the midperipheral and peripheral lens edge parameters to also fit a bit steeper.
        - Also went back to the base curve and lens diameter thing and told her about the 0.5mm change in diameter = 0.05mm change in base curve rule of thumb.
13. How do you decide what cleaning solution to give the patient?
    - Discussed MPS vs hydrogen peroxide.
    - Talked about how if the patient is a shift worker or has a poor sleep schedule, then a hydrogen peroxide or a MPS that requires long soaking time wouldn't be suitable (e.g. 6 hours aosept, boston advance conditioning solution vs. 5 minutes for optifree puremoist, 4 hours for biotrue).
    - Isnt optifree 6 hrs , and menicare plus 5 min? Ah XD i mustve confused it then - pretty sure optifree is 6hrs as well, then Medicare is 5mins
14. Other than soaking time what else would you consider? And what solution would you recommend?

    - Antimicrobial action. Recommended Optifree Puremoist as most common recommendation as it has Aldox + polyquad antimicrobials which are effective against bacteria + acanthamoeba trophozoites but not cysts.
    - Are there any solutions that are effective against cysts or is there anything that can be done to remove cysts?
        - Said not really for SCLs. For RGPs could consider autoclaving as that's the only effective method, but is not routinely done and needs specialised equipment.

GREAT WORKKKKK!!!!!

11:00am - btw make sure she can see ur desk and upper half or shell get u to move ur cam

1. R +6.00/-1.00x120 L +3.00/-2.00x110. Will this Px be suitable for CL?
   R +6.50/-1.25x120 L +3.00/-2.00x110. Yes due to R high hyperopia and significant compound hyperopic anisometropia.
2. Some questions about anisometropia. E.g. refractive vs axial, which type is better for CL, etc. sorry i forgot but not too complex questions
3. What lens will u give this px
   For a scl i would give myday toric cause high + in RE. gave the deets about myday (e.g. SiH, material, Dk/t, water content, mod etc). Also change the LE Rx to -1.75DC as -2.00 not available.
4. On fit assessment what do u look for
   Centration (1mm beyond limbus 360), movement 0.2-0.5mm, lag 0.2-0.4mm, tightness 40-60%, comfort, edge of lens (fluting, conj indentation), toric axis marking (6 oclock for myday).
5. Lens making is 10 degree temporal in both eyes
   Use lars so RE 130 axis, LE 100 axis.
6. After reordering, where do u expect the lens marking to be
   Still 10 degrees temporal
7. What if the lens was too tight on the px.
   DD usually only have 1 BC and diameter so u would usually try and swap them to smth else. But no other toric daily has powers above +6, so can either give +6.00 and undercorrect them or give rgp.
8. What are some complications with this patient cl fit in an rgp
   RE high plus so heavy centre thickness, can decentre and low ride, possible haloes glare and flare and poor vision, also discomfort from lens edge inferiorly
9. How can u fix that
   Steepen the fit. Reduce BOZR or increase LD. Or try lid attachment fit (but this involves flatter fit so increased BOZR which is the opposite of what u try do to centre it). Can also increase lens sag.

Thats allllllll good luck gang

11.15 She mde me move my cam and turn my blur off bc it obscured my desk …

1. Same rx → asked me to convert it to contact lens. New contact lens wearer.

R +6.50/-1.25x120 L +3.00/-2.00x110.

2. Would uou recommend CL in this px
    a. Yes there are some benefits tehy would have in their CL than in their spectacles.
3. What are these benefits
    a. Px is a high hyperope (especially RE) so accommodation would be less in CL than spectacles (moving plane to cornea)
    b. Convergence demand is less as they no longer have to look through base out prism at near.

> for a hyperope a tnear with spectacles do they look through bas eout or in prism
>
> **Hyperopes look through BASE-OUT prism at near when wearing plus spectacles.**
>
> Here's why:
>
> - At near, the eyes **converge**, so each eye looks through the **nasal portion** of its spectacle lens.
> - A plus lens acts like two prisms joined **base-to-base**.
> - Therefore, looking through the nasal portion of a plus lens produces **base-out prism**.

    c. Mentioned anisometropia - Spectacles will introduce relative spectacle magnification and they might be symptomatic to the difference of image size between both eyes. On the cornea there wont be RSM introduced BUT if their anisometropia is axial in origin, the spectacles were actually creating an equal image size effect (this is knapps law but i didn't mention it as i didnt want to die) making them now symptomatic in contact lenses
4. Choose a CL to fit
    a. Can i ask what they wanna wear their contacts for
5. To replace their glasses
    a. Okay, we can start in a daily lens, we can use Myday toric as it goes up to +8.00 sphere.
6. Can you use the exact script you read out?
    a. No, have to change the LE cylinder to -1.75 or -2.25. I might opt for -1.75 to avoid over minusing px and work to refine with over-refraction
7. You got the trial and put it into the patients eye, what would you look for in the lens assessment
    a. Movement (0.2-0.5mm)
    b. Make sure lens is nto too tight with push up test, normal 50%
    c. Axis marking at 6 oclock for myday, mislocation will not fully correct astigmatism.
8. What else??
    a. Limbus fully covered 1mm in all four quadrants to avoid disruption of limbal area.
9. Anything else?
    a. When they blink even if the lens does rotate it does return to its original position.
10. Anything else? (KILL ME)

    a. Centration as well.
11. Anything else?????
    a. Not to sure.
       Does she want you to say like lens edge? Comfort? Or maybe VAs? Lag? Checklist: centration, coverage, interaction with lids (interpalprebral which it should be usually), tightness, movement (lag is whatever), comfort, VA. Isn't this it?
          Yeah I think I only missed the interaction with lids… but like i feel like she wanted something else? Maybe it was VA. Cry. I think i got confused and thought she meant SLE only.
12. Whats the primary purpose for the correction
    a. Clear vision?? Oh so I would do an over refraction (i thought she was talking about slit lamp only)
13. Wat do you normally check before Over-rx?
    a. The lenses have settled.
14. What else
    a. The comfort???? (I think she wanted something about the edge)
15. Lets say you put the lenses on and the lens mislocates 10 degrees temporally what would you do.
    a. Is it for LE or Re?
16. Both eyes
    a. Apply LARS → but is it consistent location?
17. Yes
    a. Axis 130 RE LE 100
18. With the new lens what are you hoping to see
    a. It is still consistently rotated 10 degree both eyes so the effective axis is intended.
19. Why is it important to have movement with lenses?
    a. Promote tear exchange to remove debris from post-lens tear film, allow new oxygenated lenses to reach the cornea.
20. What sthe main reason limbal coverage is important
    a. Not enough limbal coverage - edge of lens irritates limbal stem cells, localised irritations
21. Why is the limbus important
    a. Stem cells are responsible for.. Their irritation can trigger release of veg-f… i think i am so lost here. (Stem cells in limbus promote corneal healing + transparency)
22. Why or why not RGPs
    a. RGP would also be useful. But the right eye centre thickness would be thicker and would be more prone to inferior decentration
23. What can you do to minimise this
    a. Increase lenns diameter and steepen the base curve
    b. Try lid attachment to hold the lens up
    c. Interpalpebral fit to hold the lens up with the lower lid…?
24. What else can you change

    a. Peripheral curves, steepen them (im not sure here)
25. What would that help
    a. Decreasing movement of lens (not sure), thus minimise decentration
26. Lets talk about reusable lenses,, why not monthly
    a. It is still a choice, for this patient as they are a new lens wearer, dailies reduce the complexity of incorporating solution and cleaning regimen. It can still work.
27. What sort of px would you go for monthly not DD
    a. Good hygiene, willing to take the time to clean the lenses properly
    b. Good sleep schedule - needs 6 hours for lens to disinfect for MOST solutions
    c. Wearing lens all the time - more economical
    d. Higher script - some higher rx are not obtainable in daily
28. How do you choose the solution you rec
    a. Any previous history of sensitivity to preservatives? → can go for a hydrogen peroxide solution but needs 6 hours to neutralise or else ocular toxicity.
    b. MPS can be useful but solutions optifree pure moist needs 6 hours still for full disinfection but wont burn ur eyes if u put it in eearly.
29. How do you choose between MPS solutions?
    a. Optifree puremoist → acanthamoeba trophozoite form ONLY not cysts, but also has polyquad for antimicvrobial agent.
30. Beside looking for disinfection agents, what else?
    a. Some solutions are better for removing some deposits rather than other
31. Some clinical signs that suggest you must change solution?
    a. SLK, described what it was and what causes it → go to h2O2 solution instead
    b. SICS → can be cause dby pres
32. What would u see if you have toxicity
    a. Diffuse epithelial staining , can have conj hyperaemia
33. Ok thats all we have time for

Sorry 33 questions?????

Question: for set up is she more concerned about seeing arms or face, cause I can't have both with my desk it's too short
I showed her my arms but then se said i cant see ur face so i had to push my laptop half off my desk on an angle lol…. ToT
Yeah she said she needs both arms and face so my laptop wa son a angle to the side
Ughhh, my table width is the size of my laptop

11:34 - Script is -6.00/-1.00x120 OD and -3.00/-2.00x110

1. Go ahead and convert that into a CL script and talk through the process please.

After applying BVD correction, script is -5.50/-1.00x120 and -3.00/-1.75x110

2. What CL are you going to recommend?

I'm going to ask some other history questions first in order to determine what the best modality would be. What is the motivation? What is the preferred wear?

3. Px has never worn CL and wants to replace CLs completely.

Generally i like to start the px on daily lenses such as Coopervision MyDay Torics if theyve never worn CLs before, but assuming the px wants to wear full time, id be happy to put the px into Coopervision Biofinity monthly lenses.

4. Alright lets stick with the monthly soft CL. The px goes ahead with those lenses and now you need to assess the CL. What does your consult look like?

I would like to wait 15 minutes for the CL to settle and then i want to check vision and fit. For fit, im gonna check centration, coverage 1mm around the limbus, lag of around 0.5mm, movement and tightness. Im gonna do push up test and aim for about 50% tightness, perhaps 40-60%. Axis marking around 6 oclock.

5. You said you want to check lag of up to 0.5mm. Does that mean you can have more lag than that?

Ideally you want 0.2-0.5mm of lag. No more cause it might be loose.

6. So if the lens is loose, what would you expect to see the px complaining about?

I would expect fluctuating vision, potentially lens falling out

7. Anything else?

Perhaps id also expect there to be irritation due to the lid interaction?

8. Good, lets move on from that. If the axis marking is 10 degrees temporal in each eye, what are you going to do?

Are the lenses consistently at that point and are not fluctuating?

9. Yes they are consistent

I would apply the LARS rule and add to the right eye and subtract 10 degrees left eye.

10. Assuming you get those lenses, where would you expect the markings now?

I would expect it at 10 degrees temporal in both eyes.

11. Good. When would you consider RGPs?

Sad face…

I would consider RGPs when the script is beyond the scope of SCLs or when the fit is not sitting well, such as when the base curve and diameter changes cant account for a toric cornea and it is jumping between 5 and 7 oclock.

12. What is the mechanism of how RGPs can correct vision when SCLs cant?

Well in something like a bitoric lens, essentially it uses the tear lens to correct corneal astigmatism.

13. So if the back surface is correcting the corneal astigmatism, what is the front surface doing?

…Umm, correcting residual astigmatism? (+ any remaining spherical refractive error)

14. Hmm…ok. Moving back to the monthly soft contact lens, what lens solution are you choosing?

That would depend on how long the px can allow the lenses to sit in a solution. Assuming that the lenses can sit in a solution for at least 6 hours, i would choose Optifree puremoist. But if the px isnt sleeping well and can only clean for say 4 hours, then id go Biotrue.

15. So what other factors influence your decision?

Well id also consider the size of the molecule used to disinfect. So in Optifree, polyquad is a relatively large molecule that cant enter the CL matrix, whereas Biotrue has biguanides which are smaller and can enter the lens matrix potentially causing toxicity.

16. What else influences your decision??

Acanthamoeba coverage. So Optifree has some coverage for Acanthamoeba, but only the trophozites not the cyst form.

17. What can kill the cyst form?

Only heat can really kill the cyst.

18. Why not use heat then?

Heat can damage and warp the lens

OK thats all for today. Thank you.

Guys lily needs to see both face and arm??? My desk is sitting really low i dont think thats possible even i pushed my laptop against the wall

What about hands?

I think she wants hands cause she wants to see ur paper

Is she fine with seeing the bottom half of my face only lol - no :((

TLDR from 9:47-10:02am (josh): you're allowed pen and paper. make sure she can see your upper torso when u set up the camera. she asks "guaranteed questions" that u can find in the contact lens resources that are pretty easy to answer.

- So are we allowed to write things down other than the rx?

write whatever u want down :)))

To those who have done the orals - how fast did it actually go by?

it went by so quick… soon enough you'll hear "okay, I think that's all for today"

Just remember that for LARS rule, the toric marking DOESN'T change after u apply LARS rule (e.g. it will still be at 5 o'clock but this is what you expect and want, because you have factored the rotation in.)

the only questions i wasnt sure about were:

1) base curves of different types of daily disposable (i said flatten the base curve in response to a tight lens, so from 8.6 to 8.7 or 8.8 and she asked which ones have that BC) (Biofinity toric BC: 8.7, AirOptics + Hydraglide BC:8.7)

2) when she asked the difference between corneal anisometropia and axial anisometropia

honestly I don't feel like you should stress yourself out with the minutia of all the possible questions she can ask, just make sure u cover your main bases with her "guaranteed questions". also remember biofinity toric xr lmao

Thanks josh!!!

Afternoon

2:00pm:

R: -6.00/-3.00 x 120
L: -3.00/-2.00 x 110
31 year old office worker

1. Convert
2. What lens to choose mainly for RE: after converting it I initially said like it depends on beca· use their converted script to -2.50DC doesnt come in DD but also not in monthlies. We can do MyDay Torics DD to start and then see how it goes. She said good ok.
3. What to check in lens assessment: basic stuff. VA, centration (1mm 360 degrees limbus covered), lens movement (0.2-0.5mm), lens lag (0.2-0.4mm), comfort for px, torix lens marking (should be at 6), lens tightness (40-60%), lens edge, lid interaction (interpalpebral)
4. Why do we want lens movement: This slight motion is actually a good thing. It allows your natural tears to flow underneath the lens, washing away debris and delivering fresh oxygen to your cornea. This circulation is important for keeping your eyes feeling fresh and healthy throughout the day. Without it, your eyes could become dry or uncomfortable. So, a lens that stays completely still is not ideal.
5. What if the CL was unstably rocking: change it to a different lens as we cannot use LARS for a rocking toric lens
6. If we picked biofinity, how would we clean it? That would depend on how long the px can allow the lenses to sit in a solution. Assuming that the lenses can sit in a solution for at least 6 hours, i would choose Optifree puremoist. But if the px isnt sleeping well and can only clean for say 4 hours, then id go Biotrue.
7. When to choose H2O2 over MPS: Allergies. Type shi.

Thats it

2:30

Same script as above.

1. What lens would u pick and why?
2. How would u assess lens fit, what are you looking for?
3. Say the lens rotates 20 degrees temporally, what would you do?
4. If the lens is not rotating consistently, what would you do-

5. Benefit a myope would have with this prescription → whats the answer for this? Increase field of view, she wanted to know about magnification is it more or less with cls? cosmesis, dont myopes benefit from increased mag - i think if axial then increase, but if refractive is decrease
    - Anisometropia and aniseikonia (** this is for both myopes and hyperopes):
        - Refractive: it would be better to switch to CLs - they see true size and avoid the unequal minification you'd get with minus spectacle correction.
        - Axial: this would be worse for them.
        - Only benefits it's for refractive type
    - "Magnification" effect – since vertex distance gone, spec mag is not there. They might like this change.
    - Cosmesis (like you said) - face/eyes won't look so minified
    - Field of view – less ring diplopia for myopes
    - Less heavy/chunky lens on specs?
    - Removes aberrations
6. Cleaning solutions for RGPs- she wants u to give a few examples for this. boston advanced cleaner and condition solution, h2o2 - aosept, oxysept, menicare plus, soleko
7. Cleaning solution for SCLs and why ur picking one over another
8. Can u tell me why u undercorrected the cyl is it ok to undercoorec the cyl, said no ideally want to give full but since patient is first time wearer would like them to be in dd rather than monthly.
9. If monthly, what lens? Said bioinfinity toric XR. What other lens, can you give me a lens? Couldnt think of any
10. Then said back to lens assessment, why do we want some movement of the lens?

YOU DID WELL CONGRATS UR DONE!!

3:00pm
R -6.00/-3.00x120
L -3.00/-2.00x115

1. Convert to cl rx
   R -5.50/-2.75x120 L -3.00/-1.75x115
2. What CL rx you would order and lens would you choose?
   R -5.75/-2.25x120 (max cyl is 2.25, and spherical equivalent)
   L -3.00/-1.75x120 (instead of 115 to account for nasal torque)
   I said DD for first time CL wearer but my dumbass said Biofinity
3. Biofinity? Are you sure?
   Oh no sorry thats monthly, i meant My Day Toric
4. Why choose DD over monthly?
   First time wearer, more convenient
   Choose Biofinity if wants better VA (wider cyl range), wearing everyday (more economical), need good hygiene and have time to clean lens for 6hrs in solution
5. Okay you choose MyDay Toric, what do you look for during fit assessment?

Centration, limbal coverage (1mm beyond limbus 360), movement (0.2-0.5), lag (0.2-0.4), tightness (40-60%), lens edge for fluting or conj indentation, toric axis mark, px's comfort and VA and over-rx

6. Where do you expect axis mark
   6 oclock
7. What would be an unacceptable for torix axis marking???? Sorry i forgot the exact question?
   Unacceptable when lens is not stable and rocks between two uh different oclocks?? I forgot what i said but she was like okay good
8. If the axis is stable at 20 temp for both eyes. What would you do?
   Reorder lens R axis 140 and L axis 100, yup
9. She was like so you order new lens then what do you do? Do you put it in their eyes (what lily???)
   Yes? 😀?
10. She giggled and was like yea okay then what would you expect with the new lens?
    Still stay at 20deg temp both eyes
11. You tried that but the px still not happy with the lens, something about not fitting right…what else can you do? Was it too tight/too loose? Why was it not fitting I think it was more like axis not stable now
    Refit lens, choose a different lens - biofinity or RGP
12. Why would you choose Biofinity over RGP
    This is when i realised i fucked up a bit but anygays
    I said soft cl would be more comfortable for first time wearer….
13. What solution to use for Biofinity?
    Optifree MPS bc it has antimicrobial coverage, kills acanthamoeba trophozoites but not cysts… Or there's also H2O2
14. Why H2O2 over Optifree and why H2O2 wasn't your first option?
    Choose H2O2 over Optifree if px is allergic to preservatives in Optifree
    Wasn't my first option bc H2O2 doesn't have acanthamoeba trophozoite coverage
15. For acanthamoeba cyst, what can kill it?
    Heat disinfection but nowadays, it's not used as often bc heat denatures/age the lens and cause warpage
16. Anything else we can use to kill cyst?
    Not that I'm aware of…
17. Okay that's all for today

3:16pm
R -6.00/-3.00x120
L -3.00/-2.00x115

1. Convert
   -5.50/-2.50x120
   -3.00/-1.75x115
2. What CLs would you choose and rx
   MyDay DD toric R -5.50/-2.25x120 L -3.00/-1.75x120 (nasal torque)

3. Fit assessment
   VA, centration, comfort, movement, lens lag, edge lift, lens tightness, rotational stability etc.
4. Rotational stability how would u assess that
   On blink where the axis goes and if px says vision is fluctuating?? Im not too sure if there's anything else she moved on after that
5. Lens is displaced 20 degrees temporally
   LARS rule so 140 for R and 100
6. Say its still moving back and forth between that axis what would you do
   Change lens and try a different one such as biofinity toric or RGP
7. Why choose biofinity over the other DD torics
   Biofinitiy because of the large range and because other DDs torics dont have the axis 140 available (im actually not too sure on the axis availability but she moved on after i said that)
8. Would this px be suitable for RGP wear
   Yes because of the high rx and cyl and for lens stabilisation and since we also sacrificing a little vision with underprescribing cyl in DDs they can get sharper vision if they want.
9. How does RGP stabilise vision
   Tear lens between back surface of the lens and the cornea that neutralises most of the cyl independent of rotation
10. Say the corneal cyl was -3.00, what design would you give them
    SPE bitoric because the corneal cyl is over 2.00D and the corneal is equal to the refractive cyl
11. How does a SPE bitoric stabilise
    The back surface is shaped to the cornea so it doenst move
12. What RGP solutions are there
    Boston Advance +Conditioner, Menicare, H2O2
13. What materials of RGP are there????????? And what parameters are u looking at - i told her honestly that i blanked and she moved on….
    I said Boston XO and then she asked what parameters which i then blanked, only managed to say oxygen permeability and wettability
14. Why choose Optifree over H2O2
    Antimicrobial cover + trophozites + if you dont leave lens in enough you wont get solution toxicity compared to peroxide
15. Back to monthly lens how would you instruct cleaning the lenses with optifree to the patient
    Lens out with clean and washed hands, rub and rinse, in case with new solution than soak for 6 hours, after wearing remove solution, rinse the case and let air dry away from bathroom on dry towel
16. What else?
    Discard contacts after 30 days and replace case every 3 months (i think its 1-3 months)
17. If a patient cant wait the 6 hours for lens to clean, what would you do

I wouldnt recommend it but u can you use optifree or menicon plus rub and rinse and put back into solution until you want to use it. - in hindsight maybe biotrue cause 4 hours instead

18. Okay thats all.

3:30PM

- R: -6.00/ -3.00 x 120 → -5.50/-2.75 x 120
- L: -3.00/-2.00 x 115 → -3.00/-1.75 x 120 (factoring in nasal torque)

1. First time CL wearer what do you give
    - Prefer DD for a first time. So MyDay Toric (have to give max cyl and remaining -0.50DC is added as -0.25DS to sph)
2. Say lens moves 15 deg temporally OU, what new axis would you give?
    - I said too much rotation so change lens TOT
3. She said what do you consider acceptable rotation and not acceptable
    - I said 5-10 acceptable ..? and then 15-20 too much (NO IDEA if its reasonable to say this). i think i interpreted this rotation to be fluctuating and not stable at that degree so i clarified if it stays there, and she said yea. so i said okay so id trial LARS first and see how it goes.
4. She changed the rotation to 10 degrees temporal, what new axis now
    - Short circuited and said 110 OU - supposed to be 130 R 110 L and she didn't say anything TOTT
5. What parameters/ properties of a lens affects rotation
    - I think earlier i said oh i'm assuming rotation means not good fit and maybe looser so choose a steeper BC (said acuvue oasys 1 day toric 8.5mm) so now she ask this. I said BC, eyelid forces, stabilisation method, cant think of anything else
        - + modulus, lens power
6. Why would you not prescribe DD?
    - Someone who wants to wear full time, highly motivated to wear CL and displays good hygiene, good ability to care for CL. otherwise DD is good for pretty much everyone
7. What makes someone suitable for RGP
    - High cyl, high corneal toricity (irregular corneas), wants very crisp vision, gets dry eye from SCL, working in very dusty environments, office work - now that i think about it you should not say dusty environment oops
8. What r solutions for RGP and SCL
    - H2O2 (examples)
    - MPS (examples)
9. Why would you choose one over the other
    - Inconsistent sleep, shift work
    - Could use MPS soaked CLs earlier if needed and wont get an acute response cf. to non neutralised H2O2
10. What would make you choose one MPS over the other

    - Someone who does a lot of water contact activities and more exposed to acanth. Can choose Optifree bc acanthamoeba trophozoite coverage
11. So does that mean they can wear CL to the beach then?
    - Absolutely not no CL to the beach or anywhere with water
12. What would you do if they HAVE to wear to the beach
    - NO CL. Rx goggles or if they HAVE to then switch to DD if not already using and air tight goggles as well. Then remove after 15 minutes or irrigate with saline/ lubricant
    - Why do you need to do that? Otherwise tonicity difference can cause lens adherence to eye and rip epithelium
13. Thats all thanks

3:45pm

- R -6.00/-3.00x120 L -3.00/-2.00x115
    - R -5.75/-2.25x120 L -3.00/-1.75x115
- First time cls wearer
    - I said id choose my day torics like everyone else i said low modulus comfort but hard to handle blush blah hygiene compliance no solutions
- Would u order this lens exactly
    - I said no account for nasal torque le so axis is 120 instead
- Cant remember exact q but like who is not suited for DDs or better options for them
    - I said high cyls, axis out of range, ectasia, dry eye or severe allergy, shift workers
- Are you saying allergies aren't suited for DDs or they shouldn't wear at all
    - If very severe shouldnt wear
- What abt if mild
    - I would recommend DDs bcs less protein build up and allergens blah not like a reusable one where theres more likely buildup and have to choose nontoxic solution
- Why are shift workers not suitable for DDs
    - Their shifts r like 24hrs + and DDs only for like 8-12hrs max so they need like extended wear lens such as biofnity xr which is approved for 6 nights 7 days wear
    - She said ok and omveod on didnt ask me abt dk/t
- What things do u check for?
    - Movement 0.2-0.5mm, lag 0.2--0.4mm, centration, tightness 40-60%
    - Shes like what else
    - Idk i said before u insert check for papillae and dry eye
    - Then shes like what r u trying to achieve w cls
    - I said good vision and comfort without gls then im like oh check vas after letting lens settle 10-15 mins and comfort
- What would u order if lenses both rotated temporally?
    - R 140 L 120 axis
- I think before this q she was asking me signs of hypoxia?
    - I began w limbal nv, epithelial microcysts, nv, …

- She cut me off and asked this :(: Are Microcysts usually from daily disposables?what are they
    - I said not necessarily ….
    - Microcysts usually associated with EW, they are necrotic cells
- What are complications from tight lens
    - I said seal and from tight lids, lack of movement, lens riding high
- She said why do u need movement
    - For tear exchange fluid whatever i didnt make a lot of sense
- Complications from loose lens
    - Limbal stem cells damage
- What will px feel or something
    - Blurry vision on blinking that clears after settling, px will feel it move on eye
- She said what else
    - Im like mechanical trauma friction can cause papilale
- I think thats it?

4:00pm
New wearer, OD -6.00/-3.00x120 OS -3.00/-2.00x115

1. Convert the Rx.
    a. I said OD -5.50/-2.75x120 OS -3.00/-1.75x115
2. What lens would you recommend? Would you adjust the CL Rx for this patient?
    a. I said Biofinity Toric (forgot to say XR lol), because of its large Rx range. And no I don't need to adjust because axis comes in 5 deg steps and comes in high cyl…

Could we do acuvue oasys 1 day for astigmatism? Axis only does to 110 i think and then 160 to 180 probably LOL but she seemed happy with my answerD: ohhhhhhhh okayyyy thanks
Sorry i rescind that it does go for up to -6.00 its only above oops thanksss

3. Let's say the lenses for both eyes were rotated 20 degrees temporally, what axis would you order?
    a. That would mean I need to adjust the cyl axes according to the LARS rule. So right eye would be 20 left = add 20 to 120 = 140, and left eye would be 20 right = subtract 20 = 95.
4. What do you hope to see when the Px tries on the new CLs?
    a. I'd hope to see the lens markings at the same positions as I previously saw (20 deg temporally OU), indicating a stable fit and the real axes are in the right spot
5. Would you expect the markings to change if you fit a different lens?
    a. Uhhh possibly due to different parameters interacting with eye differently and different lens stabilisation technology depending on what lens
6. What sort of lens is Coopervision Biofinity Toric XR? Parameters?
    a. It is a monthly SiH lens, talked about its parameters
7. What solution would you recommend for this patient and why?
    a. Optifree Puremoist due to acanthamoeba trophozite coverage with Aldox and antimicrobial property of Polyquad, relatively easy to use (she didnt probe further)
8. If the patient says he wants to wear CLs to the beach, what do you say?
    a. No, not allowed

9. Let's say the patient really loves his CLs (LMAO), what could you suggest?
    a. Goggles with DD CLs underneath + adjust Rx to max cyl and add remainder of missing cyl to the sphere
10. What complications of a tight lens could you expect?
    a. Umm SEAL…. What else? I started talking about RGP and she was like no for soft CL… i had a mind blank and said something about not enough tear exchange or movement or smth ANYWAY she smiled at the end and wished me a good afternoon<3 the time went by so so fast

4:20pm
Same Rx as above

1. Convert prescription: R -5.75/-2.25x120 L -3.00/-1.75x115
2. What do you want to know about the patient
3. What lens would you recommend
   MyDay DD toric R -5.50/-2.25x120 L -3.00/-1.75x120 (nasal torque)
4. If lens rotated 20 degrees temp how to fix
   -5.50/-2.25x140 L -3.00/-1.75x100
5. Where would lens marking be
   Stays at 20 degrees temp OU
6. How much rotation is too much 5-10 degrees *(not sure)*
7. What if the lens mark is fluctuating? How to fix? Tighten the fit by reducing base curve.
8. What would you prescribe instead then? Acuvue Oasys 1day
9. When would you not prescribe DD? Poor hygiene (prescribe nothing) ; shift worker/extended wear
10. What CL solutions would you use if you prescribed a monthly lens? MPS types, H2O2 types
11. Why one solution over the other? Soak time, action against acanthamoeba
12. If the lens was tight what would you expect? SEAL, hypoxia, debris due to stagnant tear flow

That's all we had time for ToT.

What is too much rotation?? Like >10 degrees? - I said 5-10 and she accepted it, but I'm not sure cool thanks :)

4:36pm:

Same qns as prev, the only NEW QNS she asked me:

-Tell me features of MyDay Toric (Modulus 0.4 , Dk/t 100, SiHy)

-When would you want a higher modulus? First time wearer, small aperture, needs it to be less flimsy to insert/ remove easily. What else? Couldn't think of anything :(

-Impacts of higher modulus lens? Mechanical friction, SEAL, discomfort, CLPC

-CLPC risk factors? Protein build up and mechanical trauma

-Why does protein build-up? -Poor cleaning, over-wear (like if its a monthly, they keep it for longer), sleeping in CLs, not using an intensive cleaning regimen to remove protein build up if its an RGP

5:00pm

R: +6.00/-2.00 x 110. L:+4.00/-2.00 x 115

1. If a patient comes in and tells you they're interested in switching to contact lenses what would you do
- I'll ask about their motivation for wanting contact lenses, how often they want to wear it,… then I'll convert the rx
2. Can you convert the rx
- R +6.50/-2.25 x 110. L +4.25/-2.25 x 115
3. What lens would you choose and does it come in the rx you calculated?
- My Day toric. Adjust cyl axis in LE to 120 because of nasal torque
4. Let's say they told they have tried a contact lens in the past but didn't like it what would you do
- I would ask if they tried DD or something else, if they remember what brand it was, what issue they had or if they had any complications
5. They tried a DD years ago they don't remember the brand but found it uncomfortable
- I would still try a DD because I would not be sure of the reason they didn't like it maybe the modulus was high, maybe they had complications, maybe DK/t was low
6. What lens parameters would make a lens uncomfortable?
- I talked about higher modulus being stiffer and more uncomfortable, corrected myself about DK/t how that causes corneal hypoxia but doesn't really affect patient comfort in the short term
7. What would you look for after fitting them in with the lens?
- Comfort, Va, SLE (check coverage contraction, movement 0.2-0.5, lag 0.2-0.4, tightness 40%-60%, rotation stability)
8. Why would you want to check for stability?
- Because we want the axis marking to be stable so that the cyl axis is where it should be, if it's not stable then their vision would fluctuate too

9. If marking is at 7 o'clock in right eye what would you do
- LARS rule —> Add 30 degrees to right eye
10. Where would you expect marking to be
- still at 7 o'clock
11. If marking is fluctuating what would you do
- This would mean this lens is not suitable for this patient so i would change brand
12. When would you see them next
- In 1 week check Va comfort and fit again
13. What can cause fit to change again
- Lens had more time to sit on their eye, maybe dryness, tear film, ..
14. When would you consider a monthly lens
15. Is this rx suitable for RGPs
- Yes, but talked about how high plus in R eye means thicker lens and can ride low
16. What else can happen with RGPs for this rx
- Umm, blanked out
17. How can you correct for lens riding low
- Decrease BZOR, increase diameter
18. What else can you do
- maybe peripheral edge- wasn't too sure
19. How would you change it?
20. Would you expect a plus RGP to move much or not really?
21. Talk about solutions for RGPs
22. Talk about solutions for SCL
23. Is the efficacy of hydrogen peroxide solutions and MPS similar or is one better than the other

5:15

1. Im gonna start off by giving you a script R +6.00/-2.00x110 L +4.00/-2.00x115. What can you tell me about this script
    - Well this px had compound hyperopia
2. Can you convert this script?
    - +6.50/-2.25x110
    - +4.25/-2.25x115
    - Said i would change LE axis depending on what lens i would pick
3. What would lens/rx you give and why?
    - MyDay toric (and change LE axis for nasal torque to 120)
    - Low modulus - better comfort for 1st time and less risk of mechanical complications
4. What do you check once you've put the lens in?
    - VA/over refraction
    - Centration
    - Coverage
    - Lag

- Movement
- Tightness
- Lens marking
- Rotation
- Px comfort

5. Okay lets say the lens rotates 20 degree temporally OU, what do you do?
    - LARS rule to adjust to R 130 and L 100
6. What do you expect with the new lens?
    - Marking in same position as prev (20 degrees temporal)
7. How long after would you do an aftercare?
    - One week
8. What are you looking for in the aftercare? Like why are you doing it again if you just did it in the first appt?
    - Mentioned lens will be settled in more so better representation of wear
    - If px has sensitivity to CL, we would see it e.g. papillae
    - If px gets dry eye from CL, we would see it
9. What solution would you choose?
    - OptiFree PureMoist
10. Why MPS over H2O2?
    - Easier to use as only 1 bottle, compared H2O2 lens case + platinum disk/tablet (idk if this counts as a good reason lol)
    - Less risk of toxicity from unneutralised H2O2
11. When would you choose H2O2 over MPS
    - Hypersensitivity to MPS
    - Px maintain compliance w/ rub and rinse
12. What would make you pick monthlies for 1st time wearer as opposed to dailies?
    - If they are motivated to not wear spectacles and want to wear all the time
    - If they are motivated to maintain compliance to hygiene practices (idk if these are correct answers)
13. How would you tell if a patient has good hygiene?
    - 1st time wearer: look at nails + if female look at lashes makeup debris???
    - Existing CL wearer: ask them to describe their routine w/ CL
14. Now lets say we fitted them with Biofinity monthlies, what else do you need to tell the px
    - Went through: wash hands, rub and rinse, store overnight 6 hrs
    - Discard MPS bottle after 6 months
    - Replace storage case after 1-3months
15. What solution would you recommend?
    - Menicon Menicare Plus - 5 minutes disinfection
16. What problem might the lens have if we do an RGP
    - Low riding can someone tell me why??? :(becuz high plus??? Yeah high plus lenses are thick in centre so they fall down
17. How do you fix it?
    - Increase diameter and decrease BOZR
    - Increase thickness of superior lens so more lens/lid attachment

- Don't say anything about stabilisation lol I just said stabilisation cos i ran out of things to say and she probed about it then i changed my mind cos you can't change the stabilisation of the RGP if its like a bitoric

18. Now lets say the px's corneal cyl is the same as the refractive cyl, what RGP do you give?
    - SPE Bitoric
19. How does SPE bitoric stabilise?
    - Mandell Moore fitting guide - lens matches similar to corneal toricity
    - E.g. Flat K do BOZR as 0.25 D flatter, and Steep K do 0.50D flatter
20. What material do we generally pick for RGP?
    - I mentioned no PMMA as no oxygen permeability
    - Then i said theres 1st, 2nd, 3rd gen
21. And she interrupted and said what do we use today? Why are newer materials better?
    - I said super and hyper Dk/t lenses
    - Then said they have better Dk/t
22. What else????
    - Non-ionic so doesn't attract as many deposits??
    - Better wettability??
23. What else?
    - I said girl IDKKK, then just said biocompatibility?
24. Okay thats all for today xx

HIII ALLIGATORRR IM SO DOOMED noooooooooo youre fineeeee

5:32pm

Me the entire exam:  hahahahhah

1. R +6.00/-2.00X100 L +4.00/-2.00X115
   Looking at the script, what do you think is the most suitable contact lens?
   I said I would recommend a toric lens, probably a MyDay toric?
2. Why is that? IT ALL WENT DOWNHILL FROM HERE T-T
   Cause rx comes in the prescription range idk what im meant to say here… also was like ig it depends on patient's motivation, have they worn it before
3. What can be reasons why daily disposables aren't preferred for patients?
   Px in a situation where they can't discard daily disposable? Lol maybe i should have been more specific eek. Might be wearing contact lenses full-time, so monthlies would be more cost-effective. Might not like the daily disposable like they find it uncomfortable
4. Calculate the CL power:
   R +6.50/2.25x100 L +4.25/-2.25x115
5. So what lens would you recommend

MyDay Toric but change left eye to +4.25/-2.25x120 cause nasal torque

6. What sort of lens assessment would you do?
   15-20mins settle, check VAs, over-refraction, check contact lens fit -> centration, full limbal coverage, lens movement (0.2 to 0.5mm), lens lag (0.2 to 0.4mm), push-up test/tightness (40-60%), rotation of lens indicated by axis marking. MyDay Toric should be a 6 o'clock
7. Let's say the lens marking is located at 7 o'clock. What do you do?
   I would increase axis by 30 degrees each eye. So revised script R +6.50/2.25x130 L +4.25/-2.25x150
8. When you put that new lens onto the patient's eye, where do you want the lens rotation mark to be located?
   Same spot, should be at 7 o'clock
9. What would you do if lens marking at 6 o'clock but it fluctuates?
   I think it's a poor fit so I should swap to another contact lens
10. What would you swap it to?
    I dunno I blanked and said biofinity toric??
11. Why not other daily disposables?
    I don't think there is a daily disposable that goes above +4.00D? (i am probably wrong T-T)
12. So now that you have recommended a monthly, you need some solutions! What would you recommend?
    I recommend them MPS like OptiFree :D cause it contain polyquad and aldox
13. But why not another MPS? (me in my head: lily why u ask that idk)
    uhhh now that u say that, i guess it depends on if they have used this solution in the past?
14. It's a new wearer.
    (i didn't even answer the q just went on about opti-free).
15. Do you know other MPS on the market?
    BLANKED SO HARD I COULD ONLY THINK ON BIOTRUE IN THE MOMENT
16. Is there other solution besides MPS?
    H2O2 solutions
17. When would H2O2 be better?
    MPS usually has preservatives so I would recommend it if patient has an allergy to preservatives, they have time to wait 6h for the neutralisation
18. Px has biofinity lens, what else do you want to talk to the patient about?
    Lens care hygiene -> how to clean CL with solution, wash hands before and after insertion/removal, discard CL at the end of the month and start with a fresh one. Monitor CL for breaks, deposit build-up
19. Anything specific to monthlies
    I could not give an answer oops
20. She recapped what I said and was like what else is part of the teach
    How to clean the contact lens case and advise them to discard the contact lens case every 1-3 months
21. How should they clean it?

Wash hands before doing so. Discard CL solution in case, clean CL case with solution. Leave it to air-dry with lens case face down on dry tissue in spot that's away from the bathroom

Lily: I think we have covered everything, all done for you today. Have a good night!

Woohoo congratulations Tuesday people!! GOOD LUCK WEDNESDAY PEEPS

Wednesday

Morning

Afternoon

Thursday

Morning

Afternoon